

Tue, Dec 06, 2022 9:20AM • 45:41

Interviewer 00:00

Fine. All right, I will not be using it. I will not be keeping it just for transcription purposes.

Therapist 00:07

I see. Yes. Okay.

Interviewer 00:09

All right. And just to start as well, I wanted to get a better sense of your clinical practice. I wanted to clarify what kind of couples therapy What do you primarily provide? What is the modality of your couples therapy?

Therapist 00:25

I work with Emotionally Focused couples therapy. And since I have the official permission to work, psychodynamic, I work, first of all, psychodynamic and then modified, Emotionally Focused couples therapy.

Interviewer 00:44

Interesting. And you are currently based in [Country A]?

Therapist 00:49

Yes.

Interviewer 00:51

Have always been based in [Country A].

Therapist 00:53

Yes.

Interviewer 00:54

All right. How long have you been practicing as a couples therapist?

Therapist 00:58

Oh, about [6-10] years now.

Interviewer 01:04

And do you? Are there specific types of couples that you provide your services to?

Therapist 01:12

Oh, no, I prefer different. Yeah. Sometimes, and sometimes I tend to have the impression that sometimes multi cultural, a couple of words, but I don't know, I don't make advertisements. Sometimes. Because, yes,

Interviewer 01:41

sorry. Go ahead.

Therapist 01:42

Oh, no, that's fine.

Interviewer 01:46

When times come to you, what is usually the presenting issue or their event that they seek your service?

Therapist 01:55

Ah, usually they feel distanced. And they don't know how that happens. Usually, they lost somehow.

Interviewer 02:13

And the end, do you do a formal assessment? When they come in?

Therapist 02:22

Ah, I used to hold on to this was more for structural lysing myself. Now, since I had some supervision with [name], Matt, and from [Country B] and [name] and [name] from [Country C], I feel more for confident to I mean, I have a plan have steps and and, and so on, but it's rather in my, in my head.

Interviewer 02:58

So before before this, what you were seeing as a more of a structure for yourself? What? Can you tell me more about that? Like, did you use relationship skills?

Therapist 03:10

So I did not understand what

Interviewer 03:13

relationship skills, pen and paper assessments were using the ones before?

Therapist 03:18

Ah yes, I thought that you would come up with questions like this. And I was already worried. I had some papers from the EFT trainings and then I use them and I, but but now I have a new computer and I don't even I would have to search for a longer time to find them. So I think it will, somehow Yeah, I had one which really helped me a lot, which was attachment interview. Scott, newly developed or modified or I, I had it from him, and I know many colleagues as well, who used it, which was really helpful to get an idea of the biography of the attachment.

Interviewer 04:14

But these were not these are not scales that the client themselves answered, right. This was more of an interview on your end.

Therapist 04:24

I tried different notes. I tried to hand it out to them, but I put her to talk it through with them when they were there. So the the answer set was, I mean, I mean, since they don't come from me, but for them, so I thought for the clients it was but I always thought about sending it to them before they come and but never. I don't know. I think it's too much. I think it's better when they come They tell and then they make a picture then before they come for financial reasons, because I think I don't do so much work before they come and pay. I paid so much for the trainings over the years. So no, I also want to add some work so

Interviewer 05:25

yeah, all right. Aside from the attachment interview, were there any other assessment scales that you were using?

Therapist 05:37

I don't know the name of it, there was one that was really going through the negative circle from both sides of partnership. So it was always about this. What do you think your partner thinks? What do you think your partner know at first? What does he feel? Or does he think what is his tendency to act and so it's a mentalization idea or I don't know they're probably in different technical schools different names for it, but that that to that also, I mean, I worked at through when when I tried to discover the negative cycle between the partners and I still do that but I do this now when they are there. But I don't I tried that but then I don't really I don't know when I have to read the paper when they come and then I cannot really be in contact and

Interviewer 06:50

Aside from the difficulty that you need to read the paper that you know aside from that difficulty Were there any other challenges that you had when using these skills?

Therapist 07:18

Then sometimes when I handed it out to them, and they wrote it for me, also, I turned too much into my head and not was not so much in contact with thought it was a client's and also I had problems with the timing in the in the in the sessions when I asked him to do that outside and but still sometimes I'm sometimes I can't imagine that I would give a couple of these these plants so they get it more theoretically, so then I don't have to tell them so much and I can work through at home and but difficulties No, I think it's helpful for for for them for me for the process. But it makes sense to get a good feeling was it to have some practice with them to figure out in which situation in which which is the best.

Interviewer 08:23

So that attachment interview, the negative interaction between the partners, any other any other skills that you're using.

Therapist 08:33

I used to have also, for all my patients since I also do not so much couples therapy, the main focus is rather group therapy or individual therapy. Then I had a handout for all my patients and the Beck's depress depression in when I did for personal identity. What is it called OPD interview where they were

I get the idea of they have a personality disorder. And also I had a question there for for the biography as well. But then it also too much paper and I have not only a small room and I have to keep it for 10 years, so I can scan it all and put it in the computer.

Interviewer 09:31

Yeah. Did you did you pay for any of these scales?

Therapist 09:37

Um, no. I mean, I paid for the workshops and often it was they handed it out and also I think I was invited to use it. Because yes, usually I Yeah, indirectly. Yes, but I didn't buy it in a shop

Interviewer 10:03

So I may getting a trait that these days you don't use these scales anymore.

Therapist 10:10

Sorry?

Interviewer 10:10

getting it right that these days? You don't use the scales anymore?

Therapist 10:15

Yes. Right I have? No,

Interviewer 10:19

you have it. All right. Okay. I wanted to ask as well and I touched on this in the email, that across the years, different therapists and different researchers have characterized the construct of relationship quality quite differently from each other. I wanted to get your view about it for you, what is relationship quality?

Therapist 10:57

Yes, I remember that I think I've not finished with having the answer yet. And it changed personally, for myself and in my life as well. For me, or for my clients, I think they I mean, often they, I have some impression that they want to feel hurt here. Hurt. And, and also there's a I mean, there's, they want to do their own and they don't want to develop at the same time, they want to be able to have contact when when they want to. partner wants to, but they often don't really understand how it comes so many misunderstanding and, and, and they want to be safe from being hurt. I mean, hurt meaning of being wounded and struggle was this. I mean, it's, it's always the same, I think, yes. Is that answering your question already? Or? Not quite? Oh,

Interviewer 12:27

Yes and no. So for you, right, like so serious thing about being they need to be heard, they need to be understood. You're saying that they want to develop but they still want to have contact when they want to? So for you is it's what relationship quality is? Or maybe, sorry, go ahead.

Therapist 13:01

Were you ever thought I don't know if it's helpful, but if this building trust is often a topic okay, how can they do and this being active and being passive and also within the way to see and the reality? And I think also because often the thinking is so somehow dualistic and if you are on the one side thinking one extreme then you are not focusing on the other and then the partnership, let us have the perspective it becomes like on the playground where the children go up and down and run the department sometimes it's always the opposite. But

Interviewer 13:52

Maybe to maybe to approach it from a different angle. Maybe what I'd like to have is for you, what differentiates couples with higher relationship quality and those with low relationship quality

Therapist 14:15

For me, if they are able to talk in a serious, calm way, without being afraid that the other one acts. So it's can be spoken but not has to be acted and they know that the partner can differentiate between acting and talking. And also other versus partner can feel how they feel and if they can feel if the partner can feel how they feel. That is often And if that doesn't work, often it as one can view what the other periods and the other says. No, then it's not so easy to answer. Yes. I mean, this is why they come. So I should have an answer. And often I don't even know. And yes, I haven't. You know? Sometimes I think it is getting old together. And being in a trustful relationship with someone who cares. And who's there for the for one if you if you need one. And you can reach out and respond and yeah, like the stronger bonding, normal concept. Everyone needs someone new, sometimes. It's their fault. I think it's often convinced that it's all about this. I mean, except for all these other expectations from I don't know. Tutoring together or having something together house, sailing boat or house or whatever. But that does not really add on.

Interviewer 16:42

So what would you say is the most important thing or most important things in a high quality relationship? In your professional experience?

Therapist 16:51

I think it's trust ,trust. Security giving? Wow.

Interviewer 17:01

All right.. And there's how you conceptualize your relationship, because you're talking about here like about trust about the thing that you were talking about? This how you conceptualize relationships, often differ from how your clients conceptualize relationships. And this therapy?

Therapist 17:24

Yeah, I mean, yeah, they because they want to they, I think this was in the digital space. I think it was before Emotionally Focused couples therapy, that all the concepts, as far as I heard, problem solving concepts and couples come, and they want me to solve problems. But I am, this is what I like about this approach that is Emotionally Focused couples therapy is saying, Yeah, we do that later, when we have worked out the negative cycle and decided, if you if you want to, if you want to decide to change that,

and then we can do that, but not before this. For them. It's always like, what they were making a good new experience. And if it works out, well, and often does, and actually, if you want an honest answer, I had the feeling that already has concepts were named concept, but they were not really concepts because, I mean, they were Everyone has his own ideas. And they took it from individual therapy for me, that's a bit arrogant thinking but I always had the impression that analytic family therapy, they moved to Freud say about couples therapy, not so much and so on. And so and also with cognitive behavioral and systemic and, and, and, and I also was on a conference where they said that often are these techniques techniques in the 80s or 90s, lead to that the couple divorced after that, and 90% or something like that? And that's because it was not so convincing for me this is why I was looking for something that works so that I can really have to I have the feeling that it is convincing me.

Interviewer 19:42

All right. And we also know that the relationship is a complex system, right? It involves love sex, relationship, maintenance, behavior, satisfaction, conflict, all of that. During therapy, how do you decide which one of these to focus on which one are complex system to target

Therapist 20:18

Oh, can you repeat the question please?

Interviewer 20:21

Sure, given how complex relationship is with all of its different parts, how do you decide which one to focus on? How do you decide which one of the complex system of a relationship to target in therapy?

Therapist 20:41

I know if I'm answering this question in the way you expecting, but I'm giving it a try, I usually go around this way I ask them when they feel distanced. If they can agree on a situation where they pose questions, this is usually a system, a situation where they make contact or where, where they say goodbye. Better would be both within when one comes in, from work home and so on. And then they focus of working out a negative cycle that they both can agree that this is a typical negative cycle. Yeah, which is not so into it. So it goes a bit. I don't know, it's not really answering the question, but it is part of the technique. I think, as long as I understood. Maybe you can help me if I can, then I can give it another try.

Interviewer 21:50

But I think I see where you're coming from that it's really the thing that you're focusing on in this context system are the negative cycles, they're negative patterns of interaction. So that's really where you start.

Therapist 22:05

Yeah, also. Yeah, okay. Yes. And you certainly can, if they have an agreement on that 10 common basis and I go on or also I want to find out if they want to separate or if they want to have a romantic relationship. And I also ask I say take drugs and also say, aspirins or sidesteps in the relationship, because I want to know this first. But then I..

Interviewer 22:45

do you determine when to end couples therapy

Therapist 22:55

I mean, it also has to do with German health system and so it is limited by usually they can. In the German Health System psychotherapy is covered by the by the health system. Most of the people have health insurance that is not so expensive. And they pay auto psychotherapy. And since I have a contract with insurances, often day, I charged with this and this is this is a background why I have a limit up, which is which is I don't know, I think I never did that. But theoretically, the limit would be with after 30, 32 sessions, but I've never had a couple that came that long. As far as I know, they sometimes it's limited and that is always not such a good idea. But then I cannot really say that I mean sometimes when I'm through the circuit I wouldn't, I wouldn't see them so long 10 sessions also usually 12 sessions, maybe five maximum 15 but often the couples don't come that long. And I don't know I have to see how long those cut those two couples who come see me now, because I had also a break. I didn't see it for a while and now I try to remember how long To finish the last couple always. So individually, this because I don't have so many couples can really, I didn't really figure it out yet, actually, I mean, some couples come once and then never want to come back again. Sometimes it's helpful for them sometimes. I don't know, I'm the bad guy then. And..

Interviewer 25:32

How often are these sessions once every week, once every two weeks,

Therapist 25:38

Or rather every three weeks,

Interviewer 25:42

Every three weeks? And how long one or two hours,

Therapist 25:49

Usually 90 minutes, up to 100 minutes.

Interviewer 25:57

Of currents as well, in the email, I also touched on this, but we are currently developing a relationship quality scale that hopefully better represents a relationship as a complex system that will help that aims to help therapists both in treatment planning and in treatment evaluation. So how interested would you be in a scale like this?

Therapist 26:23

That's a good idea. Yes..

Interviewer 26:27

And what would you be looking for in a scale like this? Like, how can we make sure that this scale is useful in your practice?

Therapist 26:40

Knowing how to what want to know, high escalated couple, for example, I would want to know, the control indications like drugs, drug use, and also other indicators that would be negative predictors like sidesteps or so on violence, and as far as I know, there's these three negative predictors. And so I wanted to want to know if they want to stay together or want to if they want to separate, that could be helpful if I would, or knew this before. But I also can, I mean, I can ask it, but then I have a really heavy overview what they what they want, and then it would be cheaper for them. I don't know if it would be more helpful. Probably yes. I'm not even sure about this. And also, I would want to have the attachments style, like this Scott bully interview, which is about I don't know if you know this one. It's about the childhood attachment experience. And then also in relationships, it is request. I don't know if you want to know this one. And I mean, personality disorders would be helpful if there are any, but then it will be quite a lot. I don't like long question. layers I rather would prefer here on both sides. Yeah. And for me, it would be helpful if this to be helpful. This negative cycle concept I don't know if it has to be honest. No, not really. No. It can be like has a proposal to them to read a book about emotionally couple Emotionally Focused couples therapy or something like this for the clients and then they were would be much more well prepared. But I could put it on the questionnaire on myself. PDF Yes, I think that's about this. I mean, Depression Scale could be helpful if it says very, very short. With us is to to question depression test, which is also open to us, I think. Big because once I had a couple that man was so depressed that I didn't even noticed and after six sessions or eight he told me that he is actually not really in the room. He wants I was so I was so shocked that I worked with him for four weeks. And probably no single word reached him. And it was really that, that I. Because usually then I always think I notice in the first second when they come into the room, but one really shocked me, that could be helpful. But I don't know.

[Portion Redacted]

Interviewer 33:01

All right. But no, that's really important information to know.

Therapist 33:06

ou should really ask this is important. Because reality. I mean, it makes sense. And you don't want to work for free. For two years. It makes you go to costs and so on.

Interviewer 33:20

Yeah, we'll see. We'll see. Definitely. And of course, with with the interview, like you're already, you know, helping us develop the scale. But would you be interested in further collaborating for the development of the scale?

Therapist 33:37

Yes.

Interviewer 33:20

Yes. Perfect., one of the things that if once we have the final version of the scale, we will, we will, you know, with your with your confirmation, we will be sending it to you and see how you like it, see if your feedback, and very likely as well, like, we would be giving therapists free copies of it like selected, for

example, you and we will give you free copies of it. Then after a six month, we will be asking how well it worked for you. what your thoughts were on the et cetera, et cetera. That's on Okay?

Therapist 34:15

I hope I can have I have some couples these days then. But I think they're always coming. I don't know where they're coming from. Yeah, we'd like to do that. Yeah.

Interviewer 34:30

All right. So actually, we're just wrapping up now before we before we end, I'd like to review what we have said in the session. So you said that you before your primary modality was psychodynamic and now Emotionally Focused couples therapy, a modified version of it. Can I just clarify what that modified, modified Emotionally Focused couple therapy for you? What makes it modified? What may makes it different from the usual EFT.

Therapist 35:04

I know my my theoretical background comes from psychoanalysis, group analysis and psychodynamic therapy. So, I this is I mean somehow, like a missing link or something I cannot say it gives me words it gives me technique that I don't find in psychoanalysis that I don't find in group and that makes it for couples it is the best. But I somehow I mean, first of all, I declare it to be psychodynamic and, and it is psychodynamic, because, but But I, I work with what I love to the workshops. And I combine it when it is not. Not ...

Interviewer 36:03

Yeah. All right. So it's really a combination of both psychodynamic and yeah. So your practice in [Country A] almost 10 years now. And you don't really specialize in any specific types of couples. But you do get also kind of multicultural couples. When clients come to you, the usual presenting issue is distance and they don't know what happened, but they've grown apart. And you used to do with assessment, you used to have these assessment scales, like the attachment interview by Scott wooley, the one that you don't remember the name, but it was about negative interaction, negative cycles. And you use to give as well the Beck's depression battery, the Personality Disorder, scale, OBD interview and a short biography questionnaire. But these days, you don't use it anymore for you, you just like have it in your brain, you talk through it in prison. Part of it, I believe you said that, you know, it's just like a logistical issue as well that when they give you the scales, you still need to read it to the timing with the sessions and it for you, it puts you too much into your head and not with the clients. And then we also talked about relationship quality. For you. It's really the when I asked how you define relationship quality, you went into, you know, how how relationships for relationships, like they need to be heard, they need to be understood, even though like they want to develop, they still want to connect with each other or have contact when they want to know the safety from being heard for safety from being wounded and just really the trust of building trust. And you know, not to have a dualistic thinking and for you what differentiates couples with high relationship quality and those lower relationship quality is its ability to talk ability to be calm without you know without acting negatively. The partners can feel how the other are feeling you know, growing old all together being in the trustful relationship reaching out and bonding.

Therapist 38:25

Can I have something on the topic of rupture and repair also max the high relationship quality ? Yeah, can repair the rupture?

Interviewer 38:44

Perfect. All right. So there was a rupture in your there and trust insecurity that you mentioned before as well. And then you also talked about when we talked about how you can separate your relationships and how your clients conceptualize relationships. We talked about how well basically the history of psychodynamic or for didn't say much about about, you know, proper therapy. And so for you, the EFT sort of the missing link before for you to address like problem solving concepts. But now, you know, it's before problem solving needs to like go to that stage of ending the negative interaction patterns, etc. And that's also for you, like how you, I asked how you focused on the complex system. And for you, it's really, you know, start there on the negative interaction cycles, etc. But they asked how you determine when to add a couple of therapy talked about the [Country A] health system that there are 32 sessions, but your couples don't really take that long. Usually it's 10 12 15. And how do you know that they're already good with this, like, once they've ended it like how? How do you say that you know that this was successful, the therapy was successful?

Therapist 40:19

How they know if they if they say they say that they are thankful, then I see that they can use this technique. And it's so easy. If they, yeah. Indicator.

Interviewer 40:40

All right. And then you said, Of course that you're interested in the development in the scale as well. And when I asked you how you know how to make it useful for you personally for you, you want to know about, like contraindications, for example, drug use, violence, do they want to stay together or not? Also attachment styles, possibly personality disorders, possibly depression, possibly like a short primer to EFT? But you do. But you do have thoughts about the current crisis about which we need to deliberate on later on.

Therapist 41:21

Yeah, also, I had another idea. It would be a very short conclusion of the outcome of the questionnaire. That would be great. It doesn't have to be. Could be a graph or target, was something one, two, three sentences and not many sentences. That would be great. Although they're some, yeah, I'd like, I don't know suicidality or something like this that or, drug use? Pet mark. That would be easier than it is visually, I think it is not computer based, but it is paper based or?

Interviewer 42:14

No, it would be hopefully it will be computer based just so it didn't give the automatic reports. Actually, that's that's the idea as well. So I'm glad that we are converging on that. All right. Anything else?

Therapist 42:28

Yes. Once you said that, and what means high quality relationship? I was not sure. If it was just one word missing. Because you making the conclusion fast as I was the wish that a couple not wound each

other are not being heard. And I don't have to hurt the word not. So if you look down that they want to be hurt, and I don't.

Interviewer 43:00

Yes, I got that. Yes All right. Before we end, I just like to talk to you about how we will handle the data. Of course, we want this to be as as a research project, we want it to be as open as verifiable and as transparent as ethically possible. So I wanted to know which data sharing option you were more comfortable with. One option is that the transcript portions can be shared, none of which can be identified to you. Another option, if you were bolder, is that the whole transcript can be shared, removed of sensitive and overly identifying details. So it will still be anonymized. However, in this option, there is a chance that someone who knows you professionally well can piece together the details in the transcript, like oh, he's practicing psychodynamic, or EFT. And they can narrow it down to their who among their colleagues they believe it is, but again, no sensitive information will be shared. So just to which are you more comfortable with?

Therapist 44:13

I mean the first one is absolutely okay, the second one I don't mind as well. I mean, if you if you if it's for you is better to have the second option. I also it gives you more freedom for for your work, but actually I mean I feel, and who shouldn't be, I mean, that all sound already knows. Why should you? Why shouldn't someone know? No, no, I don't. The second version is fine now.

Interviewer 44:49

Okay. So I'll be one of these months. I'll be sending you a copy of the transcript and if you want anything redacted, we will do that. All right, so I think that would be it, actually. Thank you so much, [participant]. It's been really great. It's been a really great interview. It's been great.

Therapist 45:11

Thank you [interviewer] and we go the right way [interviewer]

Interviewer 45:17

Yes, And I'll be sending your warm regards to [name] next time.

Therapist 45:24

Thank you. Yes.

Interviewer 45:27

He's about to retire. I believe he just wants to, you know, travel around these days.

Therapist 45:34

He did such a good to work. He really gave something to the society back